SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Cisplatin, lomustine and vincristine (CLV)

NCCP 00806: selected adults with high-risk medulloblastoma or another eligible primitive neuro-ectodermal tumour, including pinealoblastoma. · NCCP 00806 · version 2 · 42-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
CLV combines three chemotherapy medicines after craniospinal radiotherapy for selected adults with high-risk medulloblastoma or another eligible embryonal tumour. It is part of a specialist treatment plan; it is not a glioma regimen.

Schedule
Start four to six weeks after craniospinal radiotherapy, once cleared by your team. In each 42-day cycle, lomustine is taken once on Day 1, cisplatin is infused on Day 1, and vincristine is infused on Days 1, 8 and 15. Eight cycles are planned, with individual safety review.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Lomustine
ORAL CHEMOTHERAPY CAPSULE

- ☐ Delayed and cumulative low blood counts
- ☐ Nausea / vomiting
- ☐ Lung injury
- ☐ Kidney and liver changes
- ☐ Rare second cancers and fertility effects

Cisplatin
CHEMOTHERAPY INFUSION INTO A VEIN

- ☐ Nausea and vomiting, including after the infusion day
- ☐ Kidney injury and changes in salts such as magnesium
- ☐ Hearing loss or tinnitus
- ☐ Peripheral nerve damage
- ☐ Low blood counts, infection and bleeding
- ☐ Allergic or infusion reactions
- ☐ Fertility effects and harm to a pregnancy

Vincristine
CHEMOTHERAPY GIVEN INTO A VEIN

- ☐ Peripheral and autonomic nerve problems
- ☐ Constipation and bowel obstruction
- ☐ Jaw pain and weakness
- ☐ Tissue injury if medicine leaks outside the vein
- ☐ Hair thinning / loss

Discuss the exact diagnosis, treatment goal, evidence limits, alternatives and burdens; medicine-specific risks, monitoring, fertility and pregnancy precautions; any unlicensed supply or off-label use; and practical support. This schedule follows craniospinal RT and needs its own consent discussion.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00806 v2.

SACTCheck v0.84.39
Developed by Dr Paul O'Brien